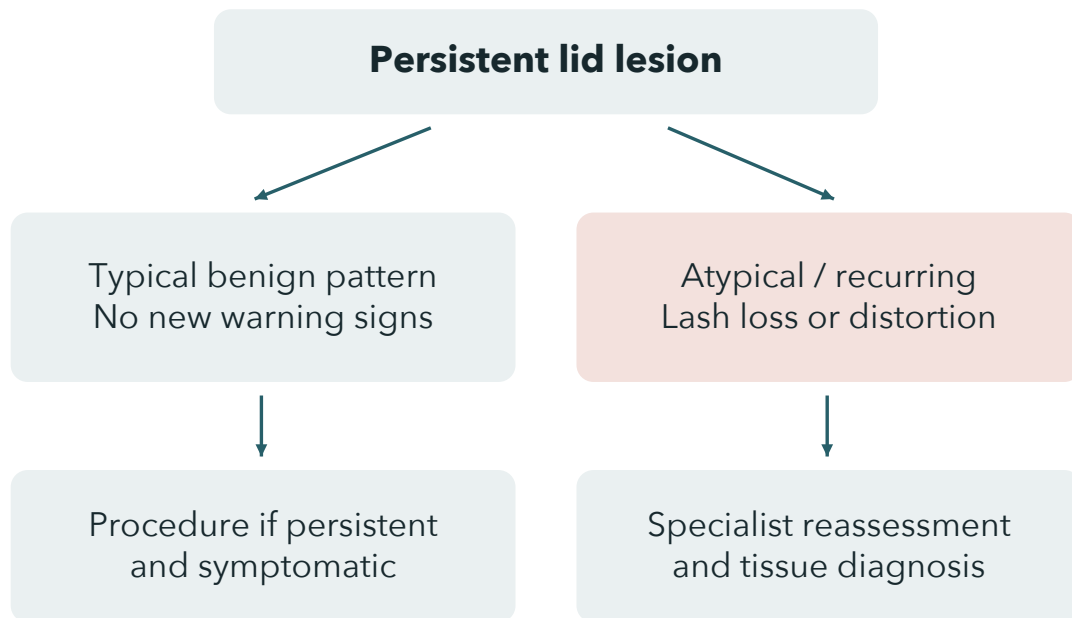


Eyelid lumps: persistence and morphology

CLINICAL TYPE B | Focused decisions for M3-M5

A persistent lump needs a revised decision

Follow the lesion, not the label



Original clinical decision schematic. A previous benign label does not overrule new examination findings.

Common benign pattern

Finding	Clinical inference
Tender lash-root pustule	External hordeolum; warm compresses, with assessment of spreading infection.
Usually painless tarsal nodule	Chalazion: sterile lipogranulomatous inflammation; initial compresses/lid care.
Typical but persistent and symptomatic	Specialist procedure may be appropriate after conservative care; antibiotics are not a routine cure for the sterile lesion.

Morphology helps prioritise the differential

Pearly edge, vessels and ulceration



Unmodified supplied Anki example labelled BCC, note 1699861730272. Appearance guides suspicion; histology confirms the lesion.

Malignant warning patterns

Finding	Clinical inference
Pearly rolled edge + telangiectasia	Suggests basal cell carcinoma; local destruction can occur despite slow growth.
Same-site recurrence, lash loss or diffuse unilateral inflammation	Consider a tumour masquerade, including sebaceous carcinoma.

Biopsy is a specialist plan

Record size, site, lid architecture and nodes, then refer for appropriate tissue diagnosis. Do not apply a blanket total-excision rule to every lesion; subtype, size and extent guide sampling and reconstruction.